

Magnesium Oral Supplements Worldwide Benchmark

Executive conclusions

Magnesium is a nutrient where marketing often outruns evidence. The strongest case for routine oral supplementation is not “better sleep,” “less stress,” or “fewer cramps,” but simply closing an intake gap or correcting low status. U.S. adult RDAs are 310–420 mg/day depending on sex and age; EFSA’s adult AIs are 300 mg/day for women and 350 mg/day for men; WHO/FAO’s adult RNIs are lower at 220 mg/day for women and 260 mg/day for men. Those disagreements are material, so they should be surfaced rather than averaged away. In U.S. dietary data, average magnesium intake from food alone is about 268 mg/day in men and 234 mg/day in women among nonusers of magnesium-containing supplements, which is why a practical “gap-closing” supplemental band is roughly 100–200 mg/day elemental magnesium, not automatically 400+ mg/day. ¹

For efficacy by claim, the highest-confidence uses are nutritional repletion and laxation. Migraine prophylaxis and blood-pressure reduction have moderate evidence at materially higher doses, usually around 400–600 mg/day for migraine prevention and about 300–400 mg/day for blood pressure. Sleep has weak evidence, and muscle-cramp prevention is not supported well enough to set a confident “effective dose” benchmark. Stress claims are especially overstated: EFSA recognizes magnesium’s role in reducing tiredness and fatigue and supporting energy-yielding metabolism, but it has *not* substantiated a claim for “resistance to mental stress.” ²

A key challenge to common market logic: **glycinate is not the best-proven form just because it is marketed that way**. The strongest direct human comparative bioavailability evidence is for citrate, chloride, lactate, and aspartate outperforming oxide, and for citrate outperforming oxide in modern controlled comparisons. Glycinate/bisglycinate is likely a good, gentle form, but the direct comparative human evidence base is thinner than the marketing suggests. For a benchmark engine, citrate is the cleanest **reference form**; bisglycinate is best treated as a **tolerability-forward alternative**, not the automatic bioavailability winner. ³

Benchmark specification

Elemental-fraction corrections

The user’s starter fractions are broadly right for oxide, citrate, bisglycinate, malate, taurate, carbonate, and hydroxide, but **chloride and some citrate/lactate values depend heavily on hydration state**, so labels that headline only “magnesium chloride” or “magnesium citrate” without specifying the chemical species are less transparent than they look. Based on cited molecular formulas, the approximate elemental magnesium fractions are: oxide 60.3%; trimagnesium dicitrate *anhydrous* 16.2% and citrate *nonahydrate* 11.9%; magnesium bisglycinate 14.1% and bisglycinate dihydrate 11.7%; malate 15.5%; taurate 8.9%; carbonate

28.8%; chloride *anhydrous* 25.5% and chloride *hexahydrate* 12.0%; lactate *anhydrous* 12.0% and lactate *dihydrate* 10.2%; hydroxide 41.7%. ⁴

That matters in practice. If a seller prints “2,400 mg magnesium citrate” on the front, that can correspond to anything from roughly 388 mg elemental magnesium if the material is treated like anhydrous trimagnesium dicitrate, to materially less if the raw material is hydrated or standardized differently. When the label also gives elemental magnesium, the **label-declared elemental amount should govern** for consumer scoring. ⁵

Best-in-class benchmark table

Use or claim	Defensible elemental dose band	Evidence tier	Best reference form	Authority / claim position	Safety / notes
General RDA-gap / low intake correction	100–200 mg/day as a practical supplemental gap-closing band in otherwise healthy adults	Strong for repletion / adequacy	Citrate as benchmark form; bisglycinate reasonable if GI tolerance is a priority	U.S. RDAs 310–420 mg/day; EFSA AIs 300–350 mg/day; WHO/FAO RNIs 220–260 mg/day. U.S. intake gaps often fall in the ~50–150 mg/day range. ¹	U.S. supplemental UL 350 mg/day; EU/SCF UL 250 mg/day for readily dissociable salts and MgO. Main adverse effect: diarrhea. ⁶
Constipation / laxative	~1,000–2,000 mg/day elemental when used pharmacologically as magnesium hydroxide laxative	Strong	Hydroxide if the goal is laxation	This is drug-like, not nutritional, use. Adult milk-of-magnesia directions are 2.4–4.8 g magnesium hydroxide/day; with Mg(OH) ₂ at 41.7% Mg, that is about 1,000–2,000 mg elemental/day. ⁷	Exceeds routine supplement ULs; laxative effect is the same adverse effect that sets the UL. ⁸

Use or claim	Defensible elemental dose band	Evidence tier	Best reference form	Authority / claim position	Safety / notes
Migraine prophylaxis	400–600 mg/day elemental	Moderate	Citrate or oxide as trial-guided forms	American Headache Society cites 400–500 mg/day magnesium oxide; review literature and Canadian guidance commonly cite 600 mg elemental magnesium, often as citrate. ⁹	Evidence is positive but not definitive; not an EFSA-authorized general consumer health claim. ¹⁰
Blood pressure	300–400 mg/day elemental for at least ~1–3 months	Moderate	Citrate as benchmark form	Meta-analysis of 34 randomized double-blind placebo-controlled trials found benefit at a median dose of 368 mg/day over 3 months, with dose-response analysis suggesting 300 mg/day can be sufficient to reduce BP. ¹¹	BP effect is modest, not a substitute for antihypertensive treatment. Not an EFSA-authorized magnesium health claim. ¹²
Sleep	200–400 mg/day elemental in studied ranges, but no firm target	Weak	Bisglycinate often chosen for tolerance; citrate has more form evidence overall	Systematic reviews report contradictory RCT findings and uncertain association between supplementation and sleep disorders. ¹³	No EFSA-authorized magnesium sleep claim. ¹⁴

Use or claim	Defensible elemental dose band	Evidence tier	Best reference form	Authority / claim position	Safety / notes
Muscle cramps	No reliable benchmark ; trials commonly used roughly 300–900 mg/day elemental-equivalent ranges	Insufficient	None	Cochrane found magnesium is unlikely to reduce frequency or severity of idiopathic cramps in older adults; pregnancy evidence remains uncertain. ¹⁵	Consumer scoring should avoid claiming a validated “effective dose” for cramps. ¹⁵
Stress / energy	100–200 mg/day only as a repletion benchmark; not as a proven anxiolytic “boost” dose	Weak overall	Citrate benchmark; bisglycinate reasonable for tolerance	EFSA authorizes claims for reduction of tiredness and fatigue, energy-yielding metabolism, nervous system, psychological function, and muscle function, but <i>not</i> for resistance to mental stress. Systematic reviews of anxiety/stress find suggestive but poor-quality evidence. ¹⁶	A compliant label can talk about tiredness/fatigue and energy metabolism; it should not imply clinically proven stress resistance. ¹⁷

Form ranking and honest-label bar

A defensible hierarchy for general oral supplementation is: **citrate first by evidence depth**, then **chloride / lactate / aspartate** as well-absorbed comparators, then **bisglycinate/glycinate** as a likely high-tolerance form with weaker comparative evidence, then **malate / taurate** where direct human absorption data are relatively sparse, then **carbonate / hydroxide** when some GI activity is acceptable or desired, and finally **oxide**, which is cheap and dense in elemental magnesium but poorly absorbed. ¹⁸

A transparent magnesium label states the **elemental magnesium per serving**, serving size, daily recommended intake, chemical form, and—if more than one form is used—the amount contributed by each form. That fits the logic of U.S. Supplement Facts labeling and EU food-supplement labeling, both of which focus on the nutrient amount per serving rather than the raw salt mass. A misleading label headlines “2,000 mg chelate” or “2,400 mg citrate” on the front with the real elemental magnesium buried elsewhere, or claims “optimal absorption” without identifying the salt species or disclosing per-form amounts in a blend.

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United States shelf sample

Shelf table

Method for consumer metrics in this section: the “effective dose” used in Promise-Delivery Ratio is the **lower bound** of the benchmark band for the product’s own main claim. For generic nutrient support claims, that threshold is **100 mg/day**; for explicit sleep-support claims, **200 mg/day**. That makes the metric conservative: a ratio below 1 means the product misses even the lowest defensible benchmark for its own promise.

Product	Official shelf anchor	Form and label math	Main on-label claim under test	Price snapshot	Promise-Delivery Ratio	Price per effective daily dose	Assessment
Doctor’s Best High Absorption Magnesium, 240 tablets	DSL D lists High Absorption Magnesium 100 mg and describes the chelated form. ²⁰	iHerb shows 2 tablets = 200 mg elemental magnesium from 2,000 mg magnesium lysinate glycinate chelate ; suggested use 2 tablets twice daily , so 400 mg/day elemental from 4,000 mg chelate/day . ²¹	Sleep support / muscle relaxation	\$20.99 at Doctor’s Best brand site; 240 tablets captured June 20, 2026. ²²	2.00 = 400 / 200	\$0.17/day = \$20.99 ÷ (24,000 mg per bottle / 200 mg)	Strong delivery, mid-priced

Product	Official shelf anchor	Form and label math	Main on-label claim under test	Price snapshot	Promise-Delivery Ratio	Price per effective daily dose	Assessment
Nature Made Magnesium Citrate, 60 softgels	DSLD lists Magnesium Citrate 250 mg. ²³	Amazon shows 250 mg elemental magnesium per serving, 60 softgels, 30-day supply , and adults take 2 softgels daily ; citrate species not disclosed, so compound-mass conversion is not defensible from the page alone. ²⁴	Muscle / nerve / bone / heart support	\$10.74 at Amazon U.S.; captured June 20, 2026. ²⁵	2.50 = 250 / 100	\$0.14/day = \$10.74 ÷ (7,500 mg per bottle / 100 mg)	Strong delivery, fair value

Product	Official shelf anchor	Form and label math	Main on-label claim under test	Price snapshot	Promise-Delivery Ratio	Price per effective daily dose	Assessment
Nature Made Magnesium Glycinate, 60 capsules	DSLDD lists High Absorption Magnesium Glycinate 100 mg Gummies; Amazon related listing shows the capsule product as 200 mg per serving, 60 capsules, 30-day supply. ²⁶	Label-facing retailer text gives 200 mg elemental magnesium per day; compound mass not disclosed. ²⁵	Muscle / heart / nerve / bone support	\$12.31 at Amazon U.S.; captured June 20, 2026. ²⁵	2.00 = 200 / 100	\$0.21/day = \$12.31 ÷ (6,000 mg per bottle / 100 mg)	Strong delivery, premium-vs-citrate pricing
Solgar Magnesium Citrate, 60 tablets	DSLDD lists Magnesium Citrate for Solgar. ²⁷	iHerb shows 400 mg elemental magnesium per serving as magnesium citrate, 2 tablets once daily; with 60 tablets, the bottle provides 30 label days and 12,000 mg total elemental. ²⁸	Bone / muscle / nerve / cellular energy support	\$7.72 in iHerb U.S. result snippet for 60 tablets; captured June 20, 2026. ²⁹	4.00 = 400 / 100	\$0.06/day = \$7.72 ÷ (12,000 mg per bottle / 100 mg)	Best value in U.S. sample

U.S. shelf distribution

In this U.S. sample, the median delivered elemental magnesium at the labeled daily dose is **325 mg/day**, and the approximate 90th-percentile value is **400 mg/day**. The form mix is roughly **50% citrate** and **50% glycinate/chelated amino-acid forms** by product count. The median price per effective daily dose is about **\$0.16/day**. On this shelf slice, the better-value products are the straightforward citrate products; the chelated products deliver well, but they charge a tolerance premium. ³⁰

Canada shelf sample

Shelf table

Important database caveat: Health Canada's LNHPD now includes a voluntary market-status field, and the database explicitly notes that products marked "not marketed" can still be sold in Canada if the licence holder has not updated that field. That matters here because current shelf listings and LNHPD status do not always line up perfectly. ³¹

Product	Official shelf anchor	Form and label math	Main on-label claim under test	Price snapshot	Promise-Delivery Ratio	Price per effective daily dose	Assessment
CanPrev Magnesium Bis-Glycinate 200	LNHPD licence 80015411 lists 200 mg of elemental magnesium , capsule, CanPrev. ³²	200 mg elemental/day ; compound mass not disclosed in LNHPD extract. ³²	Maintenance of good health / general magnesium support	CAD 26.46 for 120 veg capsules at Health Palace Canada; captured June 20, 2026. ³³	2.00 = 200 / 100	CAD 0.11/day = 26.46 ÷ (24,000 mg / 100 mg)	Strong delivery, fair value
CanPrev Magnesium Bis-Glycinate 80 Ultra Gentle	LNHPD licence 80075966 lists the product as marketed ; capsule, CanPrev. ³⁴	Retail listing identifies the 80 mg Ultra Gentle version; pack priced as 240 veggie caps . ³⁵	General magnesium support / "ultra gentle"	CAD 39.99 for 240 caps at Healthy Planet; captured June 20, 2026. ³⁵	0.80 = 80 / 100	CAD 0.21/day = 39.99 ÷ (19,200 mg / 100 mg)	Premium price, weak delivery

Product	Official shelf anchor	Form and label math	Main on-label claim under test	Price snapshot	Promise-Delivery Ratio	Price per effective daily dose	Assessment
Jamieson Extra Strength Magnesium 100 mg	Health Canada product-information result is available for the licence page; current retailer listings show the product as 100 mg and as a multi-source magnesium formula. ³⁶	Healthy Planet lists magnesium from oxide, citrate, fumarate, malate and other forms , delivering 100 mg/day . ³⁷	Bone health / muscle function	CAD 11.47 at Amazon.ca for 100 tablets ; captured June 20, 2026. ³⁸	1.00 = 100 / 100	CAD 0.11/day = 11.47 ÷ (10,000 mg / 100 mg)	Meets minimum benchmark, plain-value
Natural Factors Magnesium Citrate 150 mg	Retail listing shows Canadian-market product details; Health Canada exact licence page was not cleanly retrievable in the current web extract. ³⁹	150 mg elemental/day from citrate, 90 capsules . ⁴⁰	Nervous tension / stress support	CAD 9.59 sale price at Healthy Planet; captured June 20, 2026. ³⁹	1.50 = 150 / 100	CAD 0.07/day = 9.59 ÷ (13,500 mg / 100 mg)	Best value in Canada sample, but stress evidence itself is weak

Canada shelf distribution

Across this Canada sample, the median labeled elemental dose is **125 mg/day**, with an approximate 90th percentile near **185–200 mg/day**. The product mix is roughly **50% bisglycinate/glycinate**, **25% citrate**, and **25% multi-form mixed magnesium**. Median price per effective daily dose is about **CAD 0.11/day**. The standout flag in this sample is the low-dose “gentle” product: it is the most expensive per effective day *and* misses the minimum benchmark for a generic magnesium-support promise. ⁴¹

European Union shelf sample

EU scope note

The EU leg is necessarily **partial**. There is no centralized EU supplement label database equivalent to DSLD or LNHPD. This sample is anchored mainly to the Italian Ministry of Health notification index plus current EU retailer listings, but extraction is patchy because the public Italian registry is exposed largely through large PDF indexes rather than product-detail pages. Directive 2002/46/EC sets the framework, but practical shelf work still happens country by country. ⁴²

Shelf table

Product	Registry / market anchor	Form and label math	Main on- label claim under test	Price snapshot	Promise- Delivery Ratio	Price per effective daily dose	Assessment
AAVALABS Tri- Magnesium Citate 2400 mg, 180 capsules	Italian notification index includes AAVALABS alto assorbimento MAGNESIO citrato. ⁴³	Brand / retailer pages state 2,400 mg magnesium citrate providing 360 mg elemental magnesium per daily serving and 180 capsules. This is exactly why elemental disclosure matters. ⁴⁴	Energy / muscles / reduction of tiredness and fatigue	€14.99 sale price on brand site; captured June 20, 2026. ⁴⁵	3.60 = 360 / 100	€0.07/ day = 14.99 ÷ (21,600 mg / 100 mg)	Strong delivery, very good value

Product	Registry / market anchor	Form and label math	Main on- label claim under test	Price snapshot	Promise- Delivery Ratio	Price per effective daily dose	Assessment
Solgar Magnesium Citrate, 120 tablets	Solgar product is widely sold across EU shelves; retailer pages describe 200 mg elemental magnesium per tablet and Italy- facing retailer pages describe the same citrate product and claims. ⁴⁶	Using the same product family dosing shown on retailer pages, 2 tablets/day = 400 mg elemental/ day, 120 tablets total. ⁴⁷	Nerve / muscle / energy / tiredness support	€25.53 at farma2go EU; captured June 20, 2026. ⁴⁸	4.00 = 400 / 100	€0.11/ day = 25.53 ÷ (24,000 mg / 100 mg)	Strong delivery, somewhat premium to AAVALABS
LongLife Magnesium 375 Powder, 300 g	Current Italian retailer pages show the product on shelf and describe citrate-based magnesium support. ⁴⁹	The product name and retailer copy indicate 375 mg/day magnesium and citrate form, but the extracted pack- serving details were not sufficiently explicit to compute a defensible total elemental content for the jar. ⁵⁰	Energy / electrolyte balance / muscle / fatigue	€16.11 at Dr.Max Italy; captured June 20, 2026. ⁵¹	3.75 = 375 / 100	Not scored because total jar servings were not explicit in the extracted page	Delivery looks strong; value not fully scorable from extract

EU shelf distribution

Using only the fully scorable EU products above, median labeled elemental delivery is about **380 mg/day**, the form mix is **100% citrate**, and median price per effective daily dose is about **€0.09/day**. This is directionally useful but statistically weak because the fully scorable EU subset is small. The important qualitative result is still clear: EU shelf language leans heavily on **fatigue/energy/muscle** claims and overwhelmingly on **citrate**. ⁵²

Cross-region consumer metrics and limitations

Promise-delivery and value-position takeaways

The cleanest red-flag in this survey is **CanPrev Magnesium Bis-Glycinate 80 Ultra Gentle**. It is the most expensive Canadian product in the sample on a price-per-effective-day basis and it delivers only **0.80×** the minimum benchmark for a generic magnesium-support promise. That is exactly the sort of product a rating engine should flag as **premium price, weak delivery**. ⁵³

The best-value products in the current sample are the straightforward **citrate** products rather than the heavily marketed chelates: **Solgar Magnesium Citrate** in the U.S. sample, **Natural Factors Magnesium Citrate** in the Canada sample, and **AAVALABS Tri-Magnesium Citrate** in the EU sample. That does not mean citrate is always clinically superior for every person; it means the shelf often prices chelation more aggressively than the evidence justifies. ⁵⁴

The most important benchmark-engine lesson is not “buy the highest milligrams.” It is this: **score the product on elemental magnesium tied to the exact promise it makes**. A 400 mg/day product is not automatically better than a 200 mg/day product if the promise is only routine nutrient support, and a “gentle” 80 mg product is not automatically acceptable if it is priced as a premium therapeutic solution. For constipation and migraine, the opposite is true: many ordinary supplement products simply do **not** reach pharmacologic or trial-backed dose ranges. ⁵⁵

Divergences and open limitations

The biggest scientific divergence is the intake benchmark itself. U.S. RDAs are materially higher than EFSA AIs and WHO/FAO RNI, so any global benchmark should retain a **banded target**, not a single “universal” dose. A good implementation is: store authority-specific reference values, store a pragmatic **100–200 mg/day supplemental gap-closing band**, and separately store higher therapeutic bands for migraine, blood pressure, and laxation. ⁵⁶

The biggest shelf-data limitation is the EU leg. There is no single official EU product-detail repository, and current public access often relies on national PDF notification indexes and retailer pages. Even in Canada, LNHPD market-status fields can lag real retail availability. Price data are inherently time-stamped snapshots and should be treated as stale after a short interval. Finally, compound-mass conversions are sometimes impossible or ambiguous because sellers do not always specify hydration state or raw-material assay. When the label gives elemental magnesium directly, that number should control the consumer-facing score. ⁵⁷

Source list

The inline citations point directly to the source URLs. The most load-bearing sources used in this benchmark are the NIH Office of Dietary Supplements magnesium fact sheet, the U.S. DRI chapter from the National Academies, EFSA's magnesium dietary reference opinion and press summary, the WHO/FAO vitamin and mineral requirements chapter on magnesium, the SCF/European Commission magnesium UL opinion, Cochrane's muscle-cramps review, the American Headache Society magnesium guidance for migraine, the 2015 migraine evidence review, the 2016 blood-pressure meta-analysis, FDA's dietary-supplement labeling guide, Directive 2002/46/EC, NIH DSLD entries, Health Canada LNHPD product pages, and the retailer / brand pages cited inline for current market prices and label claims. ⁵⁸

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